GAMSA · Gender-Affirming Medicines South Australia

An inclusive service for trans and gender diverse people: tips for every clinic

For every clinician and staff member, clinical and non-clinical · gamsa.au/pharmacy

Why it matters

Trans and gender diverse people report high levels of discrimination in health care [2]. Misgendering, using a former name, invasive questions and gatekeeping can retraumatise people, stop them seeking care, and are linked with more suicide attempts [1].

Care starts at the first contact with your booking system, not at the consultation. One poor experience with one staff member can shape how a person sees the whole service [3].

Before the visit: set up the service

- [ ] Forms ask for the name the person uses (if it differs from their Medicare name), their gender in their own words, and their pronouns [1, 4]
- [ ] The gender question follows the ABS standard: "How do you describe your gender?" Man or male; Woman or female; Non-binary; a different term (please specify); Prefer not to answer [7]
- [ ] Titles include Mx, and gender can be recorded as non-binary or X [4]
- [ ] M or F markers are hidden from the person on screens, labels and equipment where possible [4]
- [ ] Name and pronouns are recorded where all staff see them before calling, treating or following up (in a notes field if there is no other) [1, 4]
- [ ] At least one toilet is open to all genders [1]
- [ ] Signs, posters and leaflets show that trans people are welcome [1, 2]
- [ ] All staff, including reception, students and trainees, are trained in names, pronouns and inclusive language [1, 3]

> **Software:** confirm it records both a chosen name and a Medicare name; if not, ask your provider.

At the front desk

- **Ask, do not assume.** How someone looks does not tell you their gender, name or pronouns [2].
- **Keep the legal name quiet.** If a Medicare claim needs it, write it down; do not say it aloud [4].
- **Explain the system plainly.** Say when the Medicare or legal name must be used for billing, and what your software cannot do [1]. Warn that some pathology forms offer only M or F [4].

In the consultation

- **Introduce yourself.** Give your name and pronouns, then ask theirs; say details can be updated at any time [1, 2].

  > **A line to use:** *"Hello, I am Sam, one of the nurses here, and I use she and her. What name and pronouns would you like me to use?"*

- **Ask only what the visit needs.** For a sprained arm, focus on the arm [2].
- **Use their words.** Use the person's own words for their body, including in results and letters [2, 4].
- **Examinations.** Explain each step and why, ask before touching, and accept a "no" or "later" [1, 2].
- **Others in the room.** Ask the person before a student, colleague or observer joins [2, 5]. Welcome a support person; if they must step out (such as for family violence screening), explain why [2].
- **Screen the body, not the file.** Anyone with a cervix is due cervical screening; self-collection is an option [8].

After the visit

- **Carry it through.** Use the person's name, pronouns and body words in results, recalls, letters, texts and referrals [4].
- **Refer with care.** Use services you know are inclusive and safe, or ask them a few questions first [3].
- **Be clear about privacy.** Say how information is stored and who can see it, and ask before linking it to My Health Record [4].

When a mistake happens

Everyone gets a name or pronoun wrong sometimes. Apologise quickly and sincerely, use the right words and move on; then fix the record or the system so it does not happen again [4]. A long apology makes the moment about you, not the person.

If a colleague gets it wrong, correct it briefly and kindly at the time, and raise it in private if it keeps happening.

> **A line to use:** *"Sorry, I meant Alex. As I was saying, the next blood test is in three months."*

Words that help

Instead of	Try	Source
Guessing, or asking “Mr or Mrs?”	**“What name and pronouns would you like me to use?”**	[1, 2]
Calling the Medicare name aloud	**Call the name they use; write the legal name down**	[4]
“That man over there”	**“The person in the blue coat”**	[2]
Choosing your own words for their body	**“What words would you like me to use for your body?”**	[2, 4]
Starting an examination without warning	**“I would like to listen to your chest next. Is that okay?”**	[1, 2]
Bringing a student in unannounced	**“A student is with me today. Is it okay if they join?”**	[2, 5]

With colleagues

- **Staff count too.** Use the name and pronouns each colleague asks for, in person and in writing.
- **A colleague’s trans history is theirs to share.** Do not pass it on, guess or ask about their body.
- **It is the law.** Gender identity discrimination in employment and services is unlawful under the Sex Discrimination Act 1984 (Cth) [6].
- **Speak up.** Jokes, gossip and repeated misgendering harm patients and staff; address them or raise them with a manager.
- **Support a colleague who is affirming their gender.** Let them lead on their name, timing, who is told, and which toilets and change rooms they use.

Keeping it going

- ☐ Inclusion is part of induction and refresher training for every role, not one person’s job [1, 3]
- ☐ A written policy covers names, pronouns, privacy and how to raise a concern, for patients and staff
- ☐ Trans patients and staff are asked for feedback, and changes are made
- ☐ Staff learn culturally safe care for Aboriginal and Torres Strait Islander trans people, who may face both racism and transphobia [1]
- ☐ Forms, software, signs and this checklist are reviewed once a year (date: _____)
- ☐ Pride in Health + Wellbeing (ACON) or its free benchmarking index has been considered [3, 9]

Where to learn more

AusPATH Standards of Care (2026) · auspath.org.au

TransHub for clinicians (ACON; endorsed by AusPATH): tips, welcoming environments, records and language guide · transhub.org.au/clinicians

Pride in Health + Wellbeing (ACON): training, benchmarking, support · prideinhealth.com.au

Pride in Diversity (ACON): workplace inclusion · prideindiversity.com.au

Indigenous HealthInfoNet: Aboriginal and Torres Strait Islander LGBTQIA+SB health · healthinfonet.ecu.edu.au

South Australia: practitioner directory · transhealthsa.com

> **For patients and families:** plain-language guides to hormone medicines, costs and supply at gamsa.au; QLife peer support on 1800 184 527 (3pm to 9pm every day).

Sources

[1] AusPATH. Australian Standards of Care for Informed Consent Gender-Affirming Hormone Therapy, version 2 (2026), pp. 15 and 18. auspath.org.au

[2] TransHub (ACON). 10 tips for clinicians working with trans and gender diverse people. transhub.org.au

[3] TransHub. Creating welcoming environments. transhub.org.au

[4] TransHub. Medical records. transhub.org.au

[5] Medical Board of Australia. Sexual boundaries in the doctor–patient relationship (2018). medicalboard.gov.au

[6] Sex Discrimination Act 1984 (Cth), sections 5B, 14 and 22. legislation.gov.au

[7] Australian Bureau of Statistics. Standard for Sex, Gender, Variations of Sex Characteristics and Sexual Orientation Variables, 2020. abs.gov.au

[8] Australian Centre for the Prevention of Cervical Cancer. Cervical screening for trans and gender diverse people (2025). health.gov.au

[9] Pride in Health + Wellbeing (ACON). prideinhealth.com.au

All checked 6 November 2026. Tips without a number are GAMSA good practice; example lines are by GAMSA. Not medical or legal advice.

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